

Patient ID: S (Chair)Period: March 30 to June 28, 2024Report Date: June 29, 2024Coverage: 1857/2184h (85%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

|                                     |  |                             |  |                          |  |         |  |
|-------------------------------------|--|-----------------------------|--|--------------------------|--|---------|--|
| Elevated Breathing<br>12 AM to 3 AM |  | Elevated HR<br>3 AM to 6 AM |  | High HR<br>12 AM to 3 AM |  |         |  |
| Vital Stat                          |  | Avg                         |  | Min                      |  | Max     |  |
| Heart Rate                          |  | 91 bpm                      |  | 60 bpm                   |  | 109 bpm |  |
| Breathing                           |  | 12 brpm                     |  | 9 brpm                   |  | 25 brpm |  |

Last 24 hours: two elevated heart rate episodes overnight; one high heart rate episode overnight.

Episodic Events (last 24h)

| Time Span     | Duration | Episodes | Condition                   | Avg | Min | Max | Comment                                                                  |
|---------------|----------|----------|-----------------------------|-----|-----|-----|--------------------------------------------------------------------------|
| 12 AM to 3 AM | 8h       | 3        | Elevated Breathing          | 12  | 9   | 25  | Check SpO2 and lung sounds; assess for fluid overload or early infection |
| 3 AM to 6 AM  | 5h       | 2        | Elevated Heart Rate (brief) | 97  | 83  | 104 | Check temp, hydration, pain; review for arrhythmia or infection          |
| 12 AM to 3 AM | 3h       | 1        | High Heart Rate (brief)     | 102 | 91  | 109 | Check temp, hydration, pain; review for arrhythmia or infection          |

Patient clinical status over 91 days from Mar 30 to Jun 28

HR

Breathing

|                                      |                                             |                                             |                                             |                                             |                                  |                                 |
|--------------------------------------|---------------------------------------------|---------------------------------------------|---------------------------------------------|---------------------------------------------|----------------------------------|---------------------------------|
| Elevated HR<br>Jun 26 to Jun 28 (3d) | Elevated Breathing<br>Apr 03 to Apr 03 (1d) | Elevated Breathing<br>Apr 10 to Apr 10 (1d) | Elevated Breathing<br>May 12 to May 12 (1d) | Elevated Breathing<br>Jun 21 to Jun 24 (4d) | High HR<br>Jun 27 to Jun 27 (1d) | Low HR<br>Jun 24 to Jun 25 (2d) |
|--------------------------------------|---------------------------------------------|---------------------------------------------|---------------------------------------------|---------------------------------------------|----------------------------------|---------------------------------|

18 episodic events Detected, spanning 42h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate, Low Heart Rate.  
Priority grade: High

| Episodic Events | Total Hours | Highest Burden      | Episodes/day | Coverage |
|-----------------|-------------|---------------------|--------------|----------|
| 18              | 42h         | Elevated Heart Rate | <1           | 85%      |

Major Findings: Sustained elevated HR (avg 97, peak 109)

| Date   | Time Span     | Duration | Episodes | Condition          | Avg | Min | Max | Comment                                                                         |
|--------|---------------|----------|----------|--------------------|-----|-----|-----|---------------------------------------------------------------------------------|
| Jun 27 | 8 PM to 3 AM  | 7h       | 1        | High Heart Rate    | 102 | 91  | 109 | Check temp, hydration, pain; review for arrhythmia or infection                 |
| Jun 24 | 2 AM to 3 AM  | 4h       | 2        | Low Heart Rate     | 43  | 40  | 46  | Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic |
| Apr 03 | 1 AM to 2 AM  | 1h       | 1        | Elevated Breathing | 26  | 26  | 26  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| Apr 10 | 1 AM to 2 AM  | 1h       | 1        | Elevated Breathing | 24  | 19  | 30  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| May 12 | 12 AM to 1 AM | 1h       | 1        | Elevated Breathing | 24  | 18  | 28  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |
| Jun 21 | 5 PM to 6 PM  | 13h      | 4        | Elevated Breathing | 28  | 15  | 44  | Check SpO2 and lung sounds; assess for fluid overload or early infection        |

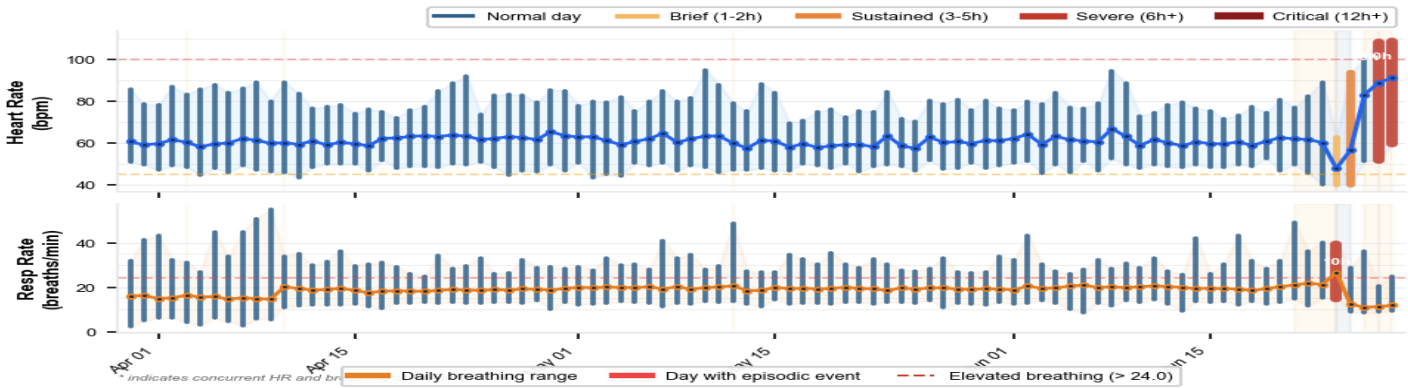
+ 1 additional condition(s) across Elevated Heart Rate; details in monitoring data.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 97 bpm, peak 106 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 28 brpm, peak 44 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 102 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 7 unstable phases with 20 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

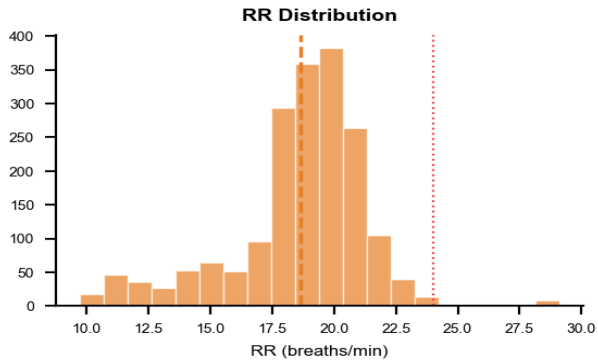
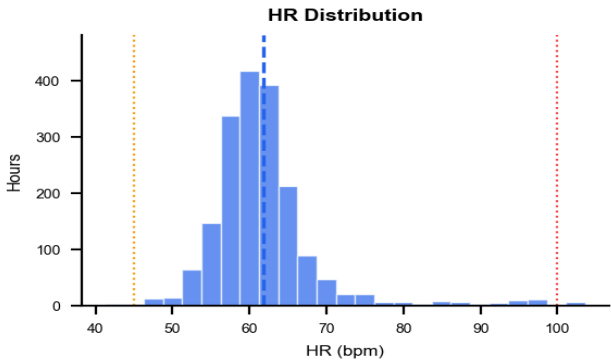
Clinical Pattern Observations:

- 1. Sustained finding: 10h continuous Elevated Breathing on Jun 24.
- 2. Clustered pattern: Episodic events on 4 of 91 days (4%).
- 3. Nocturnal pattern: 6 of 6 eligible episodes during evening or night hours.

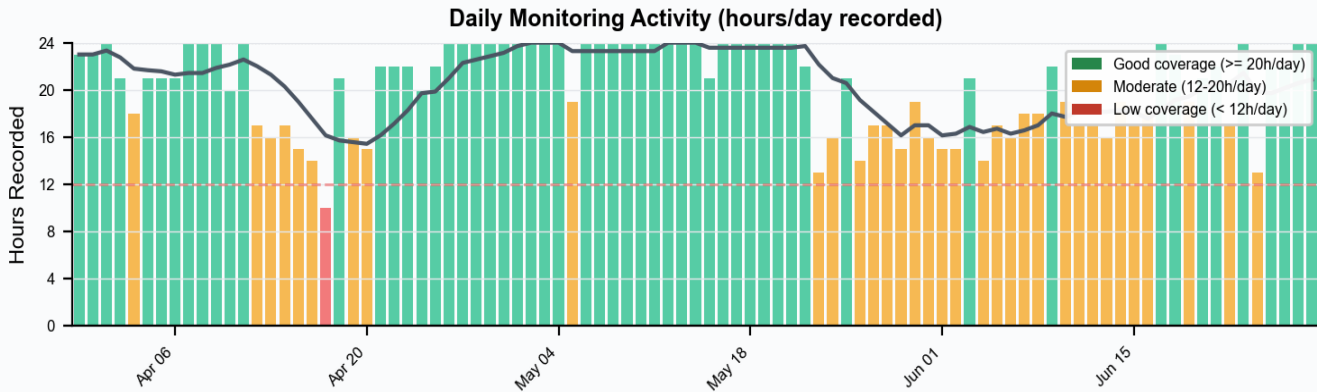


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for  $\geq 1$  hour. Episodes separated by  $\leq 1$  hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.